Day 5: the Phase 1 handoff

500 Eurasia B11 | 2 October 2026 | One recording, 2h 10m 38s, and one unique wrap-up deck

The decision this day prepares you to make

Choose the customer, problem and next experiment you will act on. Give the team usable sales materials and a shared way to track what happens.

The source deck's central instruction

Finalize the work from Phase 1 so Phase 2 can focus on experiments and movement in key metrics. The execution marathon asks for usable drafts, clear priorities and mentor feedback on the completed outputs.

The fastest workshop recap

Confirm the buyer's problem. Show the relevant task working. Connect the result to their work and goals. Explain implementation effort. Agree a specific next step with the right people.

The four deliverable groups

1. Positioning and customers: one-line value proposition, customer profiles, buyer-specific promises and qualification definitions.
2. Acquisition and sales: lead sources, an initial list, outreach and a discovery script.
3. Funnel and measurement: stages, a dashboard structure and defined KPIs.
4. Execution: three to five experiments, a one-page strategy and a demo aligned with the offer.

MediMind's working objective

Win credible hospital evaluations beyond the anchor partner. A useful next step is a qualified hospital agreeing the test, participants and review date, then completing that comparison and reaching a decision.

The supplied recording contains classroom demo practice. The wrap-up deck supplies the recap, execution assignment and program arrangements. This guide combines both sources and labels our MediMind proposals.

Sources and rewatch map

S1 means the supplied recording. D1 means the supplied wrap-up deck.

Source	Coverage	Useful navigation
S1 2h 10m 38s	Opening reflection, seven sales-demo role-plays, peer / mentor feedback and a closing discussion. All automatic transcript sections were reviewed.	06-18 min: reflection 22-35: AI adoption 36-48: demo qualification 49-60: dealer support 61-72: coaching 73-90: franchise support 91-105: app builder 106-124: recruitment 124-130: practice loop
D1 44 displayed slides	Phase 1 Program Wrap up 2 Oct. Both supplied links identify this same deck.	9-18 teaching recap 20 program timeline 21-26 Phase 2 arrangements 30 private reflection 32-42 execution marathon

Audio and video are the same session

The M4A and MP4 have the same duration and identical encoded audio-stream hashes. We transcribed the MP4 once. The recording chat contains automatic read.ai status messages, with no substantive lesson notes.

How to read the evidence

Workshop takeaway: a paraphrase of recorded practice or feedback.
Deck requirement: an instruction or recap in the wrap-up slides.
MediMind draft: a proposed application to discuss, test and refine. A written draft does not establish that customer work or an experiment has happened.

Source details

The title and program context say B11, but the cover retains 'B10'. This appears to be a template inconsistency. The video is a 640 x 360 room recording. Projected text is often too small for reliable optical recognition, so the deck and speech carry more weight than guessed screen text.

Keep the two sources distinct

The recording closes with an announcement that the Phase 1 closeout and execution marathon will follow the break. It covers the morning demo practice, while the supplied deck provides the afternoon wrap-up and assignment. Source references identify each point.

Recorded lesson: urgency has a context

Workshop takeaways | S1 approximately 06:00-18:00 | Paraphrased room discussion

What matters most

The same product can be essential to one customer and optional to another. Choose whom you serve, understand their current situation and find why the problem matters now.

Recorded point	MediMind application
Calls need a purpose, an agenda and a repeatable process. Adapt the conversation as it develops.	Before a hospital call, write the question to resolve and the next decision. After it, record what changed and improve the next call.
Customer identity and pain are fundamentals, even when revisiting them feels basic.	Name the specific hospital, buying organization, problem owner and workflow. Ask for a recent example.
The painkiller / vitamin distinction depends on customer context and urgency.	Learn where repeated work or missing information is costly enough to address now. A broad AI vision alone does not establish urgency.
Timing changes the intensity of pain. The discussion links customer profile and timing to the sales process.	Ask what changed, what deadline exists and what happens if the hospital waits. Test that trigger in discovery.
Teams need to share the new understanding, not leave it with the founder who attended.	Give colleagues the same customer definition, sales stages, demo plan and next experiment.

A better urgency question

'What makes this worth solving now, and what would you need to see to justify changing?' Use the buyer's answer to shape the demo and evaluation.

The opening discussion includes participant reflections, mentor prompts and indistinct room comments. These takeaways use the intelligible themes. Suggested MediMind wording is our application, not a verbatim quotation or a lecturer-approved message.

Demo lesson: clarity earns the next step

Workshop feedback | S1 approximately 31:00-38:00 | Practice scenarios, not customer results

The strongest correction for our pitch

A buyer's problem is not that they lack your product. Describe the operating or business result they need, then show how the product helps achieve it.

Feedback from the practice	MediMind application
A friendly opening and acknowledging the available time worked. The agenda and purpose needed to be clearer.	Explain whether this is discovery, a workflow demo or evaluation planning. Confirm time and the decision you want to discuss.
The conversation moved between theory and practical explanations. The buyer could not tell exactly what was offered.	Say what the hospital will receive and show it. Keep today's working improvement and the longer-term AI path understandable.
A new senior stakeholder needs context from the earlier conversation with their colleague.	Recap what the champion told us, ask what the buyer heard and confirm that the same pain matters to them.
Use a small, memorable structure to explain what the offer does.	Explain the current improvement, the parallel comparison and the route to adoption. Avoid a tour of every module.
'I will discuss it internally' and 'send a one-pager' leave the next step uncertain.	Check what remains unclear. Agree who needs to discuss what, and book the next useful review when appropriate.
Wanting the feature is not the customer's underlying pain.	Replace 'you need AI' with a specific problem the hospital has confirmed, such as repeated entry or delayed access to information.

A useful opening after a champion conversation

'Your operations lead described [confirmed problem]. Is that also a priority for you? I will show one way to improve it, then we can decide whether a parallel comparison is worth planning.'

The recording includes mentor and peer feedback. Role-play buyer objections are practice signals, not proof of what an actual hospital thinks. The suggested MediMind opening must be filled with a genuine discovery finding.

Demo lesson: complete the working story

Workshop takeaways | S1 approximately 38:00-59:00 | Demo-qualification and dealer-support practice

Show the result and what happens next

The buyer should see the relevant task work, understand how the result reaches their team and know what implementation requires. A description of where a feature appears is only part of that story.

Recorded feedback or example	MediMind application
The demo-qualification presenter secured a follow-up involving marketing. Feedback asked for more of the actual product interaction.	Show the hospital's agreed task working. Invite the clinical, operations or IT people needed to judge the next step.
Feedback asked the presenter to explain how leads arrive in the customer's CRM, closing the workflow loop.	Show where information is saved, who receives it and what they do next. Explain the handoff and required review.
When revenue data was unavailable, feedback suggested agreeing what information the customer could provide.	Agree which baseline and result data the hospital can supply. Distinguish workflow improvement, estimated value and realized financial results.
The dealer-support presenter asked which of two problems the buyer wanted to fix first.	Choose the hospital's first priority before showing several workflows. Keep the demonstration relevant to that answer.
The buyer asked who receives training and who provides it. The conversation clarified what the supplier would and would not do.	State MediMind's implementation work, hospital participation, training and support plainly. A free test still needs a usable plan.
Feedback praised framing the agenda, asking for additions and booking a concrete follow-up. A peer suggested positively framing the question about blockers.	Ask what looks useful and what the team still needs to verify. Surface real concerns, then agree participants, purpose and date for the next step.

A useful result question

'If your team could complete this task this way, what would change? What would you still need to check before agreeing an evaluation?'

Numbers and customer success stories mentioned in these role-plays are presenter claims, not independently verified benchmarks. They are not reused here as MediMind targets, financial projections or promised improvements.

Demo lesson: make value and trust concrete

Workshop feedback | S1 approximately 61:00-89:00 | Coaching and franchise-support scenarios

Connect the feature to this buyer's work

'Save time' becomes more convincing when the buyer can see which work disappears, how often it happens and what the team can do instead. Explore that connection with the buyer rather than assuming the value.

Lesson from the practice	MediMind application
A personalized coaching demo made the scenario recognizable. The buyer wanted users' feedback before payment.	Use approved or synthetic data that resembles the agreed hospital task. Give intended reviewers a defined comparison before the purchasing decision.
The coaching presenter forgot to arrange the next call. Feedback also warned that open-ended feature questions invite a wish list.	Book the evaluation review and ask what result users need. Prioritize development requests around agreed outcomes.
Discuss price through the value created. The coaching feedback illustrated a per-client framing.	Compare the relevant software cost and measurable workflow impact. State assumptions and avoid presenting estimates as realized savings.
In the franchise demo, the buyer asked whether the supplier would remain in business and how much setup work the team must do.	Answer reliability and capacity concerns with truthful evidence, responsibilities and a credible support plan. Explain hospital effort as well as elapsed delivery time.
The franchise buyer saw tools but did not clearly understand the value for their own role.	Explain how the workflow improvement helps the CEO, operating team and staff in their different decisions.
Feedback used missed calls and support workload to illustrate a more tangible value story.	Use that hospital's actual baseline. A missed call or saved minute is not automatically recovered revenue or better clinical outcomes.

An outcome-focused request

'What result would make this worth adopting, and which part of the current work would need to change?' Follow with a concrete test, reviewers and review date.

Numerical examples in the feedback illustrate how to frame value; they are not audited company results. Use truthful scale and capability statements. The practice discussion does not authorize inflated customer counts, invented financial evidence or guarantees about company continuity.

Demo lesson: prove the relevant difference

Workshop feedback | S1 approximately 91:00-105:00 | AI application-builder scenario

Find what the buyer could not do before

The buyer wanted to compare a new tool with one they had already tried. Feedback asked the presenter to uncover the actual failed task, then demonstrate how the new product handled it better.

Recorded lesson	MediMind application
Listening and engagement were praised. Questions still needed to uncover the buyer's difficulty more directly.	Ask for the last workflow that failed or required extra work, what staff tried and what remained unresolved.
Technical documentation and agent architecture did not explain the difference to a non-technical buyer.	For a hospital CEO, show the operating result. Explain technical architecture separately when an IT reviewer needs it.
Feedback warned about questions leading the buyer toward the answer the seller wanted.	Ask questions that can disprove the assumed pain. Record the hospital's actual answer, including reasons not to switch.
The buyer agreed to try the tool but the presenter had not established what the comparison should prove.	Before the free parallel build, define the current-system baseline, task, users and evidence of improvement.
The buyer said they could not start immediately and suggested a later review.	Agree dates with the hospital's real staff availability. Track setup, first use and decision review separately.

A better comparison question

'What could your team not complete in the current system, and what would a better result look like? Let us use that exact task for the parallel evaluation.'

Keep the competitive claim testable

Promise an agreed comparison and show the working difference. 'More advanced technology' does not, by itself, establish that this hospital gets better results.

The source scenario concerns application-building software, not a hospital deployment. The hospital questions and evaluation design are our proposed application of the feedback.

Demo lesson: start with value, state the terms

Workshop feedback | S1 approximately 106:00-124:00 | Recruitment-platform practice

Show the result before the setup tour

The buyer cared about speed, candidate quality and communication. Feedback asked the presenter to demonstrate those results with a prepared example, instead of beginning with a long job-posting setup sequence.

Recorded lesson	MediMind application
Recap the buyer's priorities and connect each demonstration to one of them.	Choose a confirmed problem and begin with the working improvement. Explain the setup after the buyer understands why it matters.
Continuous narration left the buyer watching passively. Feedback asked for relevant questions and participation.	Pause at the important result. Ask how it compares with current work and what the hospital still needs to verify.
The buyer questioned a 'guarantee': what happens if the claimed result is not reached? The seller referred to observed metrics.	Separate measured past results from a contractual guarantee. Define conditions, scope and remedy before using guarantee language.
A proposed 'trial' turned out to be a paid month, prompting clarification from the buyer.	Say exactly what the parallel evaluation costs, what is included, what the hospital must do and when a paid decision would occur.
The buyer should not have to infer why a feature meets the agreed priority.	Connect the visible task, workflow change and desired hospital result. Avoid leaving the CEO to translate a technical tour into business value.

A practical MediMind demo question

'You said this handoff creates repeated work. Here is the prepared alternative. How does it compare, and what would your staff need to test before you could judge the improvement?'

The recruitment statistics and guarantees were part of a practice scenario. They are not verified benchmarks or commitments to reuse. MediMind's free offer should be clear about the agreed test and purchasing decision, with actual terms confirmed before use.

Closing lesson: rehearse the full conversation

Workshop wrap-up | S1 approximately 124:00-130:38 | Turn feedback into the next rehearsal

The skill to build

Make the product and its value easy to explain under pressure. Practise with peers or mentors who can play a buyer, ask unexpected questions and reveal where the explanation stops making sense.

What the closing discussion emphasizes

Repeat the sales conversation, improve product explanations and prepare for objections. Use customer research and sales practice to refine the message and product. Bring difficult real questions to office hours, where mentors can help rehearse alternatives.

Pilot structure is a commercial choice

The recruitment feedback explores several ways to structure a trial or pilot and the engagement that follows. These are options, not a universal requirement. For MediMind, keep the agreed free parallel evaluation before a purchase commitment. Explain any later paid rollout separately.

MediMind rehearsal	What to check
Before the rehearsal	Choose a specific hospital persona, confirmed or explicitly hypothetical pain, current alternative and intended next decision.
During the conversation	State the agenda, confirm pain, show a prepared working result, invite the buyer's reaction and address what remains uncertain.
At the close	Agree the useful next action, participants, owner and date. Make the free evaluation and later purchasing step clear.
Afterward	Ask the observer to explain the product and value back to you. Record confusing moments and objections, revise and rehearse again.

This rehearsal checklist is our application of the recording, not a report of a completed MediMind sales call. Seek genuine understanding and informed decisions; repeated agreement is not a substitute for a buyer's concerns or evaluation evidence.

The Phase 1 ideas to retain

Deck recap | D1 slides 9-18 | Short reminders, rather than new full lectures

Topic	Point in the recap	MediMind implication
Value proposition and JTBD	Understand functional, social and emotional jobs, and match the promise to what the buyer knows.	Connect the workflow improvement to the leader's desired result and confidence in the decision.
Customer profile and research	Choose whom to interview, ask useful questions and document findings.	Treat fragmented work as a hypothesis until that hospital describes a real example.
Marketing funnel	Adapt the message to the audience and use campaign data to improve the path.	Track the steps from contact to evaluation and learn where suitable hospitals stop.
Metrics and economics	Use metrics for accountability and resource decisions. Assign an owner and interpret numbers in context.	Track delivery effort and decision outcomes alongside evaluation counts.
Experimentation	Use experiments to solve problems and improve the chances of a good decision.	Give each test a question, owner, evidence and a decision date.
Product optimization	Urgency and perceived value depend on the customer. Use the customer journey to improve conversion.	Find a workflow that matters now and help reviewers reach a useful result.
Sales process and discovery	Understand how customers buy. Specify the customer and stakeholders. Qualify leads and rehearse a useful framework.	Map sponsor, reviewers, approval path and current pain before allocating a tailored build.
Sales demo	Preparation and purposeful moments matter. Practise.	Show a relevant working task and end with an agreed next action.

JTBD means jobs to be done: what the customer is trying to accomplish. The functional job concerns the result, the social job concerns how others see them, and the emotional job concerns how they want to feel. Earlier day packs contain the detailed teaching and source examples.

Phase 2: work between mentor sessions

Historical program arrangements | D1 slides 20-30 | Schedule shown on 2 October 2026

Period / activity	What the deck says
21 September - 2 October	Foundations of Growth / Phase 1.
5 October - 27 November	Growth Weeks: experiment with the acquired knowledge and improve key metrics with mentor support.
30 November - 11 December	Fundraising phase.
10 December	Demo Day shown on the program timeline.
Weekly workload	One team stand-up, one or two mentor sessions, resident-mentor meeting weekly and other mentor hours as needed. The slide describes three to four hours available per company each week, plus guest and visiting mentors.

Program actions in the deck

Check the program site for the latest schedule. Book mentor office hours through the program's Calendly links. Complete assignments and ask for help when blocked. The 2 October reminders include the end-of-phase survey, recurring resident-mentor meetings and digitizing flip-chart work.

Reflection exercise

Slide 30 asks for a private letter to yourself to open at graduation. Choose the goals, encouragement or reflection that matter to you. This is a personal exercise, separate from customer evidence or the company's metric report.

MediMind application: a useful mentor meeting

Bring one account, one sales artifact or one experiment with a specific decision to make. Show what you tried and what happened. Leave with a concrete change, an owner and a date to review the result.

These are the arrangements shown in the supplied deck, not a live check of office-hour availability or future events. Use the current program schedule when booking. A later schedule change can supersede this historical snapshot.

The execution-marathon assignment

Deck requirement | D1 slides 32-42 | The original 2 October afternoon schedule

Block	Time	Deliverables
Setup and alignment	14:30-14:40	Agree who works on which outputs and where drafts live.
1. Positioning and customer	14:40-15:10 30 minutes	One-line USP, refined personas / ICP, customer value propositions, MQL and SQL definitions.
2. Acquisition and sales	15:10-15:45 35 minutes	Lead-generation plan, initial lead sources or segments, outreach draft and discovery-call script.
3. Funnel and tracking	15:45-16:15 30 minutes	Funnel stages and flow, dashboard structure and KPI definitions tied to the funnel.
4. Experiments and plan	16:15-16:45 30 minutes	Three to five experiment priorities, one-page strategy and aligned demo page / sales demo draft.
Validation and sign-off	16:45-17:00 15 minutes	Five-minute internal check, mentor feedback and updated deliverables.

The quality standard

Positioning should be clear in ten seconds. Acquisition work should suit an early-stage budget. Funnel definitions should stay simple. Experiments should be actionable the following week and connected to the earlier work.

The operating instruction

Work through the drafts, decide, use AI where it speeds the work, and ask an available mentor for feedback when blocked. The deck discourages rebuilding everything or spending the session discussing one detail.

Completion evidence

For each output, keep the actual draft, its owner and unresolved questions. Record mentor feedback only after a mentor has reviewed it. The following pages supply MediMind working drafts, not evidence of mentor sign-off or completed customer validation.

Block 1: positioning and customers

MediMind working draft | D1 slides 34 and 38 | Validate in hospital conversations

One-line value proposition

Test a tailored hospital system that connects your teams and brings AI into daily work, free in parallel with your current software before you decide to switch.

Initial ideal customer profile

Start with reachable Georgian private hospitals or groups that describe recurring work between disconnected teams, have a leadership reason to fix it now, and can assign an evaluation sponsor and reviewers. Confirm the actual buying organization and a supportable initial scope.

Person	Value to discuss	Evidence they need
CEO / owner	A clearer way to run the hospital and compare a replacement before committing.	Recognizable pain, operating impact, credible delivery and a purchasing path.
Clinical / operations lead	Less repeated work and more usable information at a difficult handoff.	Staff can complete the agreed task, with appropriate review and manageable effort.
Finance lead	A testable improvement in reconciliation, billing corrections or reporting work.	Comparable baseline and results, with estimates distinguished from collected revenue.
IT / data owner	A feasible evaluation and a manageable route to adoption.	Scope, responsibilities, dependencies and the actual hospital effort required.

The customer's three jobs

Functional: make the confirmed workflow work better.
Social: be seen as a leader who improves the hospital responsibly.
Emotional: feel confident about the decision and regain control over repeated problems. The latter two are hypotheses to explore, not assumed buyer feelings.

Keep the broader connected hospital and future AI vision in the conversation. Use a demonstrable current result to make the first decision concrete. Quantified savings, revenue or patient-outcome claims need evidence before becoming promises.

Block 1: qualification and buying roles

MediMind proposed definitions | MQL and SQL are gates for how we use sales time

Stage	Evidence required	Next action
Target account	A named hospital or buying group plausibly fits the chosen profile. Record a real contact route and a reason to investigate.	Research and approach.
MQL	Fit is supported, a relevant contact engages and a qualifying conversation or meaningful next step is agreed.	Run discovery.
SQL	Discovery confirms recurring pain, urgency, a sponsor, a buying path, feasible scope and a dated evaluation-planning step.	Plan the comparison.
Evaluation ready	Hospital and MediMind agree scope, reviewers, success criteria, necessary approvals and a decision-review date.	Allocate the build.
Completed evaluation	Named reviewers perform the agreed comparison and discuss evidence with a recorded proceed, revise or decline decision.	Act on the decision.

MQL means marketing-qualified lead. SQL means sales-qualified lead. These are our proposed operating definitions. They should reflect what the team can observe and write down.

Different people can move different steps

A **connector** provides a route into the hospital. A **champion** helps coordinate colleagues and explains the internal process. A **sponsor** owns the problem. An **approver or buyer** authorizes the next commitment. Technical and clinical reviewers assess their parts of the comparison. Confirm who fills each role.

Before free development

Ask: what are we comparing, who will test it, how will we judge it, when will we review it, and what decision follows? The hospital can commit to participation and a review without committing to buy.

A friendly reply, warm introduction or access milestone cannot establish all these conditions. Keep missing evidence visible and record why a hospital pauses or declines.

Block 2: acquisition and the lead list

MediMind working draft | D1 slides 34 and 39 | Founder-led and realistic for current capacity

Source	Approach	Evidence to collect
Existing professional network	Ask a clinician, adviser or hospital contact for an introduction tied to a confirmed or plausible operating problem.	Who knows whom, whether an introduction is possible and why the topic may matter.
Targeted direct outreach	Research one hospital, its relevant leader and a genuine reason to talk. Use a brief question about the problem.	Fit, current trigger, work contact route and what the recipient actually says.
Permission-based anchor proof	Use an approved reference or specific documented workflow example where it is relevant.	Permission, comparability and what the evidence proves.
Follow-up on existing interest	Return to the agreed action or a concrete unanswered question.	Next action, owner, date and reason for delay.

One row per buying organization

Record hospital / group ID, segment, fit evidence, source, connector, buyer, possible champion, pain and its evidence, stage, last contact, next action, owner and date. Multiple contacts at one hospital remain one account. Several sites may share one purchasing decision.

A small initial list

Use the next ten accounts the founder can realistically research and approach. Rank confirmed pain and reachability. Ten is a suggested working batch, not a course requirement or a sales forecast. Add hospital names only after verifying them and removing overlap with the anchor relationship.

Output still needing founder input

This page defines the list and source strategy. It does not invent hospital interest, personal relationships or named qualified leads. Fill actual account rows, then choose the first outreach batch with an owner.

Block 2: outreach and discovery

MediMind drafts | Adapt the wording to one real hospital and the buyer's preferred language

A short introduction request

'Who at [Hospital] owns the problem of staff repeating work between disconnected systems? We can build an agreed parallel alternative for their team to evaluate free before any purchase commitment. If this matters to them now, would you be comfortable introducing us?'

A first message to the hospital

'Does your team still re-enter information or chase updates between systems? MediMind connects the workflow and adds AI assistance in everyday work. We build a tailored parallel version free so your team can compare it before deciding to switch. Would a short conversation about one workflow worth testing be useful?'

Discovery step	Question
Understand current work	Describe the last difficult handoff. What happened, who was involved and what did the team do next?
Find impact and urgency	How often does it happen? What effort or consequence can you quantify? Why address it now?
Map the decision	Who owns this problem, who would test an alternative, and how would a purchase be approved?
Agree what better means	What would you need to see in a comparison to consider changing the system?
Choose the next step	Is a scoped planning session useful? Who joins, what must they bring and when can we meet?

Offer and timing

Explain the free evaluation and hospital participation plainly. If the next-ten-client price-match offer is active, specify eligibility, comparable cost items, period and scope in the proposal. Agree delivery timing only when the evaluation prerequisites and capacity are clear.

These messages and questions are drafts. Use the hospital's answers to decide whether to advance, revisit later or stop. The general question about repeated work should not become an assertion about a hospital you have not interviewed.

Block 3: the hospital funnel

MediMind working flow | D1 slides 34 and 40 | One shared account record

Step	What advances the hospital	Record
1. Target and route in	A supportable hospital and credible introduction or direct approach.	Fit and source.
2. Engaged contact	A relevant person agrees to a useful qualifying conversation.	Contact and date.
3. Qualified opportunity	Pain, urgency, sponsor, buying path and feasible next step are confirmed.	Discovery evidence.
4. Relevant demo	Buyer sees the agreed task and decides whether further evaluation is useful.	Objections and next step.
5. Evaluation ready	Scope, baseline, participants, approvals and review date are agreed.	Readiness date.
6. Parallel system delivered	The agreed test environment works and reviewers can begin.	Delivery acceptance.
7. Comparison completed	Reviewers test, discuss evidence and record proceed, revise or decline.	Results and decision.
8. Paid adoption	Commercial agreement, rollout and meaningful use progress. Track collection separately.	Contract, use and cash.
9. Retention and expansion	Review realized value, renewal and potential expansion or references.	Value and renewal evidence.

Keep permission, delivery and buyer value distinct

Data access can make an evaluation ready. Delivering the system shows your build is complete. A completed comparison shows the hospital has evaluated it. A paid agreement shows a commercial decision. Timestamp each event separately.

A hospital can pause, decline or return to discovery at any stage. Record the reason and next action. Use the buying organization as the unit throughout the funnel.

Block 3: metric and dashboard definitions

MediMind proposal | The reporting window and the sales-cycle deadline are different choices

Primary metric: completed new-hospital evaluations

Count unique non-anchor hospital buying organizations completing their first defined parallel-system comparison within a rolling 28-day window. Require named reviewers, evidence from the agreed test and a recorded proceed, revise or decline decision. Review weekly.

Diagnostic	Definition	What it helps decide
Qualified opportunities	Accounts with discovery evidence meeting the SQL definition.	Whether targeting and conversations create feasible evaluations.
Demo to readiness	Evaluation-ready accounts / accounts that held a relevant demo, tracked in the same cohort.	Whether the offer and next-step process work.
Ready to delivered	Accounts receiving the agreed usable environment / accounts ready to begin.	Whether delivery can meet the agreed scope.
Delivered to completed	Accounts reaching the evidence review / delivered evaluations in the same cohort.	Whether reviewers participate and reach a decision.
Time to first useful result	Calendar days from evaluation readiness to the first reviewer-confirmed useful workflow result.	Where setup or early use needs support.
Completed to paid	Signed paid agreements / completed evaluations in the same cohort. Track collected payment separately.	Whether the comparison creates a buying decision.
Evaluation effort	Founder, engineering, implementation and support hours for each comparison.	How many evaluations the team can support.

A simple weekly view

Show counts at each stage, new entries, transitions, age of open accounts, overdue next actions and decision reasons. Show the count beside every percentage. Keep anchor activity separate and display pending cases.

This uses the Day 4 definition of a completed evaluation and 28-day reporting window. Some earlier drafts used a first hands-on test and 30 days. Adopt one definition for future reporting, and label any historical figures measured differently. No current results or target counts are supplied here.

Block 4: experiment priorities

MediMind proposed backlog | D1 slides 34 and 41 require three to five actionable items

Priority / question	Small test	Evidence and decision
1. Can we earn a meaningful introduction?	Ask the next five appropriate contacts for one specific hospital introduction.	Record fit, relevant introductions and conversations held. Continue the source if it reaches suitable buyers, or change the request / audience.
2. Does the workflow message create a useful conversation?	Use one concise message about a verified problem with a small, researched account batch.	Record replies, held discovery and confirmed pain. Revise the message if suitable recipients misunderstand or reject the assumed problem.
3. Can a qualified buyer prepare an evaluation?	Use a one-page scope, reviewers and decision-date agreement with the next suitable opportunity.	Identify which readiness condition stalls. Confirm the feasible change before building.
4. Can users complete the promised task?	Compare one defined workflow in current and parallel systems, with intended reviewers.	Observe completion, time, corrections and help. Continue only if the agreed comparison establishes useful improvement.
5. Does the review produce a decision?	Book the evidence review when the evaluation is planned. Reconfirm the decision process halfway through.	Record participation and proceed / revise / decline. Adapt the review if it leaves unknowns or no owner for the next step.

The minimum experiment record

Question, assumption, target accounts, method, owner, start / review dates, effort limit, evidence and a continue / change / stop rule. Choose the highest current bottleneck and check that the sample and capacity suit the question.

A draft backlog is not five experiments running

Select one primary experiment first. Keep the others queued, with dependencies visible. Small samples can expose friction and improve the next action. They do not establish a reliable population-wide conversion rate.

The batch sizes above are suggested working limits, not lecturer benchmarks. Add real owners and dates before execution. Do not send outreach or allocate free engineering merely because the draft exists.

Block 4: a one-page MediMind strategy

Completed discussion draft | Dependencies and founder decisions remain visible

Decision	Working choice
Current business problem	Winning hospital evaluations outside the anchor relationship.
First customer	A reachable private hospital / group with recurring workflow pain, a sponsor and people able to evaluate.
Promise	A tailored connected hospital system with useful AI assistance, tested in parallel before deciding to switch.
First proof	One hospital-confirmed workflow comparison, using a baseline and intended reviewers.
Acquisition route	Specific warm introductions and targeted founder outreach to researched accounts.
Evaluation offer	Free within the agreed scope, with no purchase commitment before testing. Confirm readiness and a review date.
Commercial proposal	Current software price matching for the next ten eligible clients if active, with comparable scope and terms defined.
Measure	Unique new hospital buying organizations completing a defined comparison in 28 days, plus conversion and delivery effort.
First experiment	Choose the biggest present constraint: route in, message, readiness, task value or decision review.
Review rhythm	Weekly account review, documented learning and specific mentor feedback on the next decision.

What the team must choose

Name the first accounts, the workflow worth testing, the delivery owner, available evaluation capacity and dates. Confirm the price-match and development scope before quoting commercial terms. Replace hypotheses with evidence as hospital conversations happen.

Phase 2 success evidence

Keep a record of who progressed, what users tested, what changed in the task, what decision followed and what it took to deliver. An evaluation that ends with a clear decline can produce learning. Track positive decisions and paid adoption alongside completion.

The demo and page must make one promise

MediMind alignment draft | D1 slide 41 includes a demo page / sales demo

Suggested page opening

You know your hospital could work better.
Let us prove the alternative.

One tailored system connects your teams and brings AI into daily work. We build an agreed parallel version free, so your staff can test the improvement before any purchase commitment.

Clear call to action

Plan my free parallel evaluation
Start with a conversation to choose the workflow, participants and success criteria. If the next-ten-client offer is active, add: 'Current software price matched for our next 10 eligible hospital clients, within agreed scope.'

Page expectation	What the demo should show	Next decision
A connected workflow	The actual task and handoff this hospital cares about.	Is the improvement worth a comparison?
Useful AI assistance	A working relevant task, with inputs, output and required review explained.	Where would staff use and verify it?
A tailored parallel evaluation	Scope, participants, prerequisites and evidence review.	Can we agree an evaluation-planning step?
Price and ongoing development	Terms tied to supportable scope and service commitments.	What must the buyer know to judge a paid rollout?

A short demo sequence

Confirm the current priority, recap the real process, show the strongest relevant working result, ask what changed for the buyer, address the remaining concern and agree an owner and date for the next action. Allow the buyer's answers to change the sequence.

This is a proposed message and demonstration plan. It does not change the public website. Verify actual capabilities and active commercial terms before using it with a buyer.

Assignment completion and validation

One checklist for the four blocks | D1 slides 38-42

Required output	Draft in this guide	What remains to validate
USP / one-liner	Positioning page	Buyer understands the promise and it matches the available product.
Personas / ICP and customer value propositions	Positioning page	The chosen hospital and stakeholders confirm the pain and desired result.
MQL + SQL definitions	Qualification page	The team adopts observable gates and records evidence consistently.
Lead-generation plan and initial lead sources / list	Acquisition page	Fill named accounts, verify contact routes and assign outreach ownership.
Outreach and discovery drafts	Outreach page	Tailor and rehearse before actual conversations.
Funnel flow, dashboard structure and KPIs	Funnel and metrics pages	Choose one metric definition, add stage dates and review real accounts.
Three to five experiment priorities	Experiment page	Select a first test, add people, dates, effort limit and a decision rule.
One-page strategy	Strategy page	Resolve the first customer, capacity and commercial-term decisions.
Demo page / sales demo draft	Demo alignment page	Demonstrate a working workflow and get feedback on a rehearsal.
Mentor validation and updated deliverables	This checklist	Have a mentor review the actual artifacts. Record their feedback and the resulting edits.

Five-minute internal review

Can everyone explain the customer, promise and next step? Does the outreach lead to the discovery and demo we prepared? Do stage gates have evidence? Does the primary metric match the bottleneck? Can the first experiment be run with the available people and time?

Use your next mentor call well

Show the artifact and the decision that is unresolved. Ask for a concrete change, then update the draft. This guide prepares the work for review. It does not establish that Brianna or another mentor has approved it.

Sources and folder guide

Private learning pack | Prepared 8 October 2026

Recording

S1: GMT20261002-054458_Recording_640x360.mp4, 2h 10m 38s. Original folder: Desktop / Recordings / week2day5. The matching M4A has identical encoded audio. Original recording chat is retained in Working files.

Deck

D1: 500 Eurasia B11 - Phase 1 Program Wrap up 2 Oct, 44 displayed slides. Both supplied links identify this same source. All accessible text and relevant timeline / execution diagrams were reviewed. Download, copy and print were disabled, so the folder contains the link and original notes.

Method and limitations

Local automatic speech recognition, source-deck review, sampled screen images and practical synthesis. Automatic wording can contain recognition errors. Check the recording before quoting. Reported examples and presenter claims are not independently audited results. MediMind drafts remain proposals until the team tests them.

One folder, reusable formats

01 PDF: this study guide.
02 PDF: the complete automatic transcript.
Transcripts: searchable TXT, timestamped SRT and structured JSON.
Source decks: source link and access notes.
Working files: reusable study notes, source review, processing and verification.

Use this pack for authorized private study. The supplied sources include classroom company examples. No website deployment, outreach or cloud-document edits were performed as part of this request.